

UPMC Mercy

1400 Locust St, Pittsburgh, PA 15219

DISCHARGE SUMMARY Document #: DIS-003

Patient & Admission

Name	Tamara Higgins
Date of Birth	1970-10-04
Phone	412-555-5947
Address	374 Jackson Rest Greensburg, PA, 15601
MRN	MRN-496631
Admit Date	2026-05-12
Discharge Date	2026-05-16
Attending	Emily Mcguire, MD
Attending NPI	9593103416

Insurance

Primary Payer	Cigna
Primary Member #	U531003309

Primary Discharge Diagnosis

Acute STEMI, anterior wall

ICD-10: I21.A1, I25.10

Hospital Course

The patient is a 55-year-old male who presented to the emergency department with acute anterior wall ST-elevation myocardial infarction. He reported sudden onset of severe substernal chest pain radiating to the left arm and jaw while at home, accompanied by diaphoresis and dyspnea. Initial electrocardiogram demonstrated ST elevation in the precordial leads V1 through V4 with reciprocal changes inferiorly. Troponin I was markedly elevated at 8.2 ng/mL. The patient was immediately taken to the cardiac catheterization laboratory where angiography revealed a thrombotic occlusion of the left anterior descending coronary artery. Primary percutaneous coronary intervention was performed with successful placement of a drug-eluting stent to the LAD with restoration of TIMI 3 flow. Post-intervention angiography showed excellent stent apposition with minimal residual stenosis.

Following the acute intervention, the patient was admitted to the coronary care unit for continuous hemodynamic monitoring and medical optimization. Transthoracic echocardiography on hospital day two revealed an ejection fraction of 40 percent with anterior wall hypokinesis and mild mitral regurgitation. B-type natriuretic peptide was elevated at 385 pg/mL. The patient was initiated on guideline-directed medical therapy including high-intensity statin, dual antiplatelet therapy with aspirin and ticagrelor, metoprolol, lisinopril, and eplerenone. He remained hemodynamically stable throughout the remainder of his hospitalization with no

recurrent chest pain, arrhythmias, or mechanical complications. Serial troponins peaked at 12.4 ng/mL and subsequently trended downward.

The patient was discharged on hospital day four in stable condition with close outpatient cardiology follow-up arranged for repeat echocardiography in six weeks to reassess ventricular function and guide further therapy optimization. He was counseled extensively on medication adherence, cardiac rehabilitation enrollment, and lifestyle modifications including smoking cessation and dietary sodium restriction.

Procedures Performed

Coronary angiography	CPT 93458
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Medication Changes

Atorvastatin 80 mg daily	STARTED
Lisinopril 10 mg daily	STARTED
Metoprolol succinate 50 mg daily	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	9 days
Urgency Tier	HIGH
Clinical Flags	recent-revascularization, EF<40%

Dictated and electronically signed by:
Emily McGuire, MD
NPI: 9593103416
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